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Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy and
National Coordinator for Health Information Technology
Department of Health and Human Services
HHS Health
Sector AI RFI, Mary E. Switzer Building
Mail Stop: 7033A, 330 C Street SW
Washington, DC 20201

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Re: Request for Information – Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

On behalf of the American Academy of Sleep Medicine (AASM), we appreciate the opportunity to comment on the use of artificial intelligence (AI) in clinical care. As clinicians who diagnose and manage sleep disorders every day, we see both the promise and the practical challenges of AI firsthand. In sleep medicine, AI has the potential to improve diagnostic accuracy, expand access to care, and support earlier identification and treatment of common and costly conditions. However, from a clinician's perspective, meaningful adoption will only occur if AI tools are reliable, transparent, independently validated, and clearly aligned with patient safety and clinician accountability.

These comments reflect the experience of more than 9,000 AASM physician members and the teams working in over 2,400 AASM-accredited sleep programs. Our members care for patients across diverse practice settings, including hospital-based labs, independent sleep centers, and office-based practices, many of which serve Medicare beneficiaries. Across these settings, clinicians consistently emphasize that AI must function as a clinical support tool—not a black box that introduces new risks or uncompensated workload.

What are barriers to innovation, and how can HHS support the private sector?

Currently, several significant barriers hinder the responsible and sustainable implementation of AI in clinical care.

1. *Persistent uncertainty regarding medical-legal liability*. Clinicians remain unclear whether responsibility for errors, adverse outcomes, or inappropriate recommendations generated by vendor-developed AI tools rests with the treating physician or the technology developer. This ambiguity creates unacceptable professional and malpractice risk, particularly when clinicians have limited insight into how an algorithm was trained, validated, or updated. Without clear allocation of accountability,

many physicians are understandably reluctant to rely on AI outputs in clinical decision-making, even when tools may offer potential benefits.

2. *Lack of transparency and independent validation*. Many AI systems function as "black boxes," providing little or no disclosure regarding training datasets, population representativeness, validation methods, known limitations, or real-world performance across diverse clinical settings. This lack of explainability prevents clinicians from assessing whether an algorithm's outputs are reliable for individual patients or whether performance may vary across demographic or clinical subgroups. In sleep medicine, where diagnostic determinations frequently drive long-term or lifelong therapy decisions, including durable medical equipment (DME) prescriptions and chronic disease management, uncertainty about the reliability of algorithmic outputs raises significant patient safety concerns. Clinicians must be able to understand and defend the rationale for clinical decisions, and opaque systems undermine this fundamental professional responsibility.
3. *Operational and workflow challenges further complicate implementation*. AI tools are often introduced without sufficient integration into existing electronic health record systems or clinical processes, resulting in fragmented workflows, duplicative documentation, and increased administrative burden. Rather than reducing workload, poorly designed tools may add steps that require clinicians or staff to manually reconcile data, verify outputs, and correct errors. These inefficiencies diminish potential benefits and contribute to frustration and burnout among healthcare professionals.
4. *Reimbursement presents a substantial barrier.* Many clinicians are concerned that payers may assume that AI automatically delivers efficiency gains and therefore reduce payment rates or increase productivity expectations, despite the reality that safe AI use often requires additional oversight. In practice, when AI outputs are unreliable, incomplete, or inconsistently formatted, clinicians must spend additional unbilled time reviewing results, reconciling discrepancies, validating data accuracy, and documenting clinical reasoning to meet regulatory and liability standards. This time-intensive oversight represents uncompensated labor and shifts costs from technology developers to frontline providers. For small, independent, and community-based practices operating on narrow margins, these added burdens may make adoption financially infeasible.
5. *The nature of many AI systems introduces concerns about consistency and durability*. Algorithms that update over time without clear version control, locked validation, or re-certification processes may produce different outputs for the same clinical scenario, complicating longitudinal care and making it difficult to compare results over time. Such variability undermines clinician confidence, complicates quality measurement, and creates challenges for regulatory compliance and clinical documentation. Healthcare providers require stable, validated tools whose performance characteristics are predictable and reproducible.

What can HHS do via regulations to incentivize the use of AI in clinical care?

To address the aforementioned barriers to implementation, in support of safe and effective adoption, we urge HHS to establish clear regulatory and financial frameworks that reflect how care is actually delivered by implementing the following:

1. We recommend that AI vendors bear legal responsibility for performance errors attributable to their algorithms. Clinicians should not be held liable for defects in tools they did not design, train, or control.
2. Transparency requirements should move AI toward a "glass box" model, with clear disclosure of training datasets, validation processes, intended use cases, and independently verified performance metrics. From a clinical standpoint, this information is essential for informed use at the point of care.
3. Independent validation and standardized benchmarking should occur prior to deployment and at regular intervals thereafter, as clinicians need confidence that tools perform consistently across patient populations and practice settings.
4. Interoperability and validity across vendors are critical for comparing tools, preventing vendor lock-in, and supporting continuity of care when systems change.

Meaningful adoption of AI will also require *direct financial support and reimbursement structures* that recognize the real-world costs of implementation. Integrating validated AI tools requires significant upfront and ongoing investments, including software, licensing fees, IT infrastructure, cybersecurity protections, workflow redesign, staff training, and maintenance. For many physician practices, particularly small, independent, and solo practices, these costs are substantial and may be prohibitive without dedicated reimbursement pathways, grants, or implementation assistance. HHS should therefore establish *clear mechanisms to reimburse the clinical time and resources required to appropriately use, monitor, and validate AI outputs in patient care*.

Additionally, HHS should ensure that reimbursement is not reduced or restructured downward based on assumptions that AI automatically creates efficiencies. Payment models should instead recognize AI as a clinical decision-support tool that augments, rather than replaces, physician expertise and should ensure that clinicians are compensated for the time and professional judgment required to safely integrate these technologies into care delivery.

What are "novel" legal barriers that AI creates in the clinical space?

AI introduces novel regulatory and legal considerations with direct implications for clinical practice.

1. FDA clearance processes that rely predominantly on vendor-supplied validation may create uncertainty when independent verification of safety and effectiveness is not feasible.
2. Clinicians require clear regulatory guidance on whether updated or adaptive AI systems require a renewed review and how material changes in algorithm performance are disclosed.
3. Limited explainability further complicates informed consent, clinical documentation, and shared decision-making, thereby increasing the risk of liability.
4. If AI use becomes the standard of care, practices that lack the financial resources to adopt such tools may face disproportionate legal exposure.

Clear and consistent federal guidance delineating vendor accountability and clinician responsibilities is essential to mitigate these risks and support equitable implementation.

Where does the government have the most influence?

The federal government plays a central and indispensable role in shaping a trustworthy, accountable, and sustainable AI ecosystem for clinical care. Because healthcare delivery operates within highly regulated frameworks for payment, privacy, and safety, federal policy decisions directly influence whether AI tools are adopted responsibly or prematurely without adequate safeguards. The U.S. Department of Health and Human Services (HHS) is uniquely positioned to accelerate safe adoption by establishing clear, enforceable standards that prioritize patient safety, reliability, and accountability while avoiding unnecessary administrative complexity.

1. *HHS should develop and implement regulatory frameworks that require independent validation of clinical AI tools, standardized performance metrics, and ongoing post-market monitoring to ensure consistent safety and effectiveness across care settings and patient populations.* Establishing interoperability standards and common technical requirements will promote competition, reduce vendor lock-in, and enable providers to integrate AI solutions without costly system overhauls. Clear disclosure expectations regarding intended use, limitations, characteristics of training data, and version updates are also essential to enable clinicians to make informed decisions about when and how to rely on AI-supported outputs in patient care.

2. *Robust privacy and data governance protections are equally critical*. Federal policy should ensure that patient data are used solely for authorized clinical or quality improvement purposes, are not sold or repurposed for commercial gain without explicit consent and are protected by strong cybersecurity safeguards. Transparent data stewardship requirements, consent standards, and accountability mechanisms are necessary to maintain public trust and prevent misuse. Without these protections, both patients and providers may hesitate to participate in AI-enabled systems, undermining their potential benefits.

Through clear standards for safety, transparency, interoperability, liability, and data protection, HHS can create a stable regulatory environment that encourages responsible innovation while ensuring that new technologies enhance clinical care and patient trust.

How and where can advancing interoperability improve the AI market for clinical care?

Advancing interoperability is fundamental to realizing the full clinical utility, safety, and scalability of AI-enabled technologies. Without common technical standards and the ability for systems to communicate seamlessly across platforms, AI tools risk becoming siloed solutions that fragment care, increase administrative burden, and limit their practical value to clinicians and patients. *Interoperability enables independent verification of performance across environments, promotes transparency, and allows providers to evaluate and compare tools based on consistent, objective criteria*. This comparability is essential for building clinician trust and supporting evidence-based procurement decisions.

From a market perspective, interoperability also promotes competition and innovation by reducing vendor lock-in and lowering switching costs for healthcare organizations. When providers are not tied to proprietary systems, vendors must compete on quality, safety, usability, and price rather than exclusivity. *This dynamic can reduce costs for practices and health systems while accelerating improvements in tool performance and reliability*. For smaller practices, interoperable solutions are particularly important because they minimize the need for costly custom integrations and enable adoption without wholesale replacement of existing infrastructure.

Clinically, effective interoperability strengthens care coordination and continuity. *Seamless data exchange between AI tools, electronic health records, and other health information systems allows clinicians to access complete and accurate patient information at the point of care, share insights with patients and referring providers, and reduce duplicative testing or unnecessary procedures*. This capability is especially important for longitudinal conditions, such as sleep disorders and other chronic diseases, where consistent tracking of diagnostic and treatment data over time is essential to safe and effective management.

Standardized definitions, benchmarking methods, and validation criteria will ensure that outputs are meaningfully comparable across tools and vendors, enabling regulators, purchasers, and clinicians to assess effectiveness and safety on equal terms. Federal leadership in establishing and enforcing these standards will be critical to creating a cohesive, competitive, and trustworthy AI ecosystem that supports high-quality clinical care.

What challenges and concerns do patients and caregivers have with the adoption of AI technology?

Patients and caregivers have expressed significant concerns regarding the integration of AI into clinical care, particularly related to transparency, bias, privacy, cost, liability, workforce impact, and the preservation of the patient-clinician relationship. Many patients are unaware of when AI tools are used in their care or how algorithmic outputs influence clinical decision-making. Without clear disclosure and meaningful explanation, trust in both the technology and the healthcare system may be undermined.

Concerns about equitable performance across diverse populations and the protection of sensitive health information are especially prominent. *Patients expect assurance that AI systems are independently validated, free from bias, and subject to strong privacy safeguards that prevent unauthorized use or commercial exploitation of their data*. Questions about potential cost implications and accountability for errors further influence public confidence.

To address these concerns, *federal policy should establish clear disclosure standards, require independent validation and equitable performance evaluation, and ensure robust privacy protections*. Patient-facing education and transparent accountability frameworks will be essential to building and sustaining trust in AI-enabled clinical care.

Are there specific research areas that HHS should prioritize to accelerate the use of AI in clinical care?

HHS should prioritize research that rigorously evaluates whether AI demonstrably improves clinical care and workflow from the frontline clinician perspective. *Studies should assess measurable impacts on clinician burden, time efficiency, patient access, and burnout, while ensuring that AI does not shift additional uncompensated work or legal risk onto providers*. Research should also examine real-world implementation, including workflow integration, oversight requirements, and patient safety outcomes.

Targeted pilot programs in high-prevalence, high-cost conditions, such as obstructive sleep apnea, may offer meaningful opportunities to evaluate clinical effectiveness, cost savings, and scalability. In parallel, *HHS should explore payment models that support responsible, evidence-based adoption of AI, recognizing that sustainable reimbursement structures are essential for widespread implementation*.

AI holds significant promise to enhance care delivery; however, adoption will depend on demonstrated safety, transparency, independent validation, clear accountability, and appropriate financial support. By prioritizing these research areas, HHS can promote responsible innovation while ensuring that clinicians can confidently integrate AI into patient care.

Thank you for your consideration of these comments. The AASM appreciates the Agency's efforts to receive input into accelerating the adoption and use of AI as a part of clinical care . We encourage the Agency to adopt the recommended changes summarized in this letter. Please feel free to contact Diedra Gray, AASM Director of Quality & Health Policy, at dgray@aasm.org or 630-737-9700, for additional information or clarifications.

Sincerely,

Anita V. Shelgikar, MD, MHPE
President, American Academy of Sleep Medicine